Fax cover sheet

Silver Oak Community Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Brian Castillo, LCSW
Phone: 555-0186
Date: 08/04/2026 10:05
Pages (including cover): 12
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

38F abdominal wall abscess s/p I&D, now on oral antibiotics, wound healing. Independent. Hoping for d/c 08/05/2026.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

IHH Recuperative Care Referral Form

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Silver Oak Community Hospital (fictional)
Department/Floor: 3 South Med-Surg
Social Worker/Case Manager/Nurse: Brian Castillo, LCSW
Phone: 555-0186
Email: bcastillo@silveroak.example
Nursing Station: 555-0187
Authorized By: Dr. A. Brandt

Patient Information

Name: Rosa Doe
MRN: SO-7724519
Birth Date: 02/03/1988
Gender: F
Ethnicity: Hispanic or Latina
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English / Spanish
Housing Situation:
Currently Unhoused [X]
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Abdominal wall abscess s/p I&D
Admit Date: 07/31/2026
Any Wounds: Yes [X] No [ ]
Number/Location/Size/Stage: 1 - RLQ abdominal wall I&D site approx 2 x 1.5 cm, dry gauze daily
Independent with Wound Care: Yes [X] No [ ]
Home Health Ordered: Yes [ ] No [X]
PPD/TB Test or Chest X-Ray: Date: 07/31/2026 Result: CXR clear

History of Health & Living Situation

O2: Yes [ ] No [X]
Self Administer All Meds: Yes [X] No [ ]
Continent of Bowel & Bladder: Yes [X] No [ ]
Colostomy/Ileostomy: Yes [ ] No [X]
Foley Catheter: Yes [ ] No [X]
Diabetes: Yes [ ] No [X]
Insulin: Yes [ ] No [X]
Diabetes Oral Meds: Yes [ ] No [X]
Active Infection: Yes [X] No [ ] abscess - on PO doxycycline
Ambulatory: Yes [X] No [ ]
Assistive Device: Yes [ ] No [X]
Hospice: Yes [ ] No [X]
Car: Yes [ ] No [X]
Service Animal: Yes [ ] No [X]

Behavioral and Mental Health Status

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Rosa Doe
DOB: 02/03/1988
Member ID: OCH-30871246

Referring Provider

Facility: Silver Oak Community Hospital (fictional)

Clinical

Primary Diagnosis: Cutaneous abscess of abdominal wall
ICD-10: L02.211
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 08/05/2026
Requested Days: 14

Plan Use Only

Status: Pending review

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Patient face sheet

Silver Oak Community Hospital (fictional) - PATIENT FACE SHEET

Patient: Rosa Doe
MRN: SO-7724519
Account #: 99724519
DOB: 02/03/1988
Sex: Female
Preferred Language: English / Spanish
Ethnicity: Hispanic or Latina
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 07/31/2026 16:20
Admit Type: Inpatient
Unit/Bed: 3S-318
Attending: Anja Brandt, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical - page 1

HISTORY AND PHYSICAL

Date: 07/31/2026 18:00

CC

Painful swelling of lower abdomen x 6 days, fever

HPI

38 y/o F, unhoused (staying in a tent along a riverbed), presents with enlarging painful red lump on R lower abdomen that started as a "pimple". Subjective fevers and chills x 2 days. Denies IVDU, no skin popping. No abdominal surgery history.
ED: T 101.3F, HR 106, BP 118/72, WBC 15.2. Bedside US: 4.2 x 3.0 cm subcutaneous fluid collection RLQ abdominal wall, superficial to fascia, no intra-abdominal extension.

PMH

- Obesity
- Prior skin boils (no hospitalization)

Meds PTA

None

Allergies

Penicillin (rash)

Social

Tobacco 5 cig/day. Occasional EtOH, last drink >2 weeks ago. Denies drugs.

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History and Physical - page 2

Exam

Gen: uncomfortable, AOx4
Abd: soft, non-tender except RLQ abdominal wall: 6 x 5 cm area of fluctuance with 9 cm surrounding erythema, no crepitus

Assessment/Plan

Abdominal wall abscess with surrounding cellulitis, SIRS criteria on arrival.
1. General surgery consult for I&D
2. Vancomycin IV pending cultures (PCN allergy)
3. Wound culture at I&D; blood cultures x2
4. MRSA nares screen ordered
5. Dispo: unhoused, CM for recuperative care
Signed: Anja Brandt, MD 07/31/2026 19:10

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General surgery procedure note

GENERAL SURGERY - PROCEDURE NOTE

Date: 07/31/2026 21:30
Procedure: Bedside incision and drainage, RLQ abdominal wall abscess
Anesthesia: local lidocaine 1%
Findings: 4 cm subcutaneous cavity, 15 mL purulent material, no fascial involvement
Specimen: wound culture sent
Packing: 1/2 in plain packing strip placed
Plan: packing removal 08/01, then wet-dry or dry gauze as wound granulates. No further surgical intervention anticipated.
Signed: P. Iwu, MD (General Surgery)

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Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5

Date: 08/04/2026 08:15
S: Pain 2/10. Afebrile since 08/01. Eating well.
O: T 98.4F HR 80 BP 116/70 RR 16 SpO2 99% RA. AOx4.
Wound RLQ: 2.0 x 1.5 x 0.4 cm, clean granulating base, no packing needed, erythema resolved to 1 cm periwound.
A/P: Abdominal wall abscess s/p I&D, improving.
- Vancomycin IV stopped 08/03; transitioned to doxycycline 100 mg PO BID x 7 more days (through 08/10)
- PIV to be removed before d/c
- Dry gauze dressing daily
- Wound culture: Staphylococcus aureus, susceptibilities pending (see lab)
- Blood cultures no growth
- Gen surg clinic f/u 08/12/2026
Anticipate d/c 08/05 if placement accepted.
Signed: Anja Brandt, MD

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Nursing note

NURSING NOTE

Date: 08/04/2026 07:00
Pt AOx4, calm and cooperative.
Pt ambulating independently in hallway, no assistive device.
Showered independently. Toileting independently, continent.
Dressing change RLQ: pt removed old gauze, cleaned with saline and applied dry gauze and tape herself with RN observation. Technique appropriate.
Pt self-administered doxycycline 100 mg PO at 06:00 with RN observation; states schedule twice daily with food and water.
Signed: J. Okafor, RN

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Case management note

CASE MANAGEMENT NOTE

Date: 08/04/2026 09:40
Pt unhoused, no family able to house. Agreeable to recuperative care.
Home health referral for weekly wound check sent 08/03 to Valley Bloom Home Health (fictional) 555-0188. Status 08/04: awaiting agency acceptance.
Gen surg clinic f/u 08/12/2026 scheduled.
Doxycycline prescription to be filled at bedside pharmacy on day of discharge.
Signed: Brian Castillo, LCSW

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Vitals, labs and microbiology

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
08/02 20:00 | 98.9 | 84 | 120/74 | 16 | 99% | RA
08/03 08:00 | 98.5 | 82 | 118/72 | 16 | 99% | RA
08/03 16:00 | 98.7 | 80 | 114/70 | 16 | 98% | RA
08/04 00:00 | 98.2 | 76 | 112/68 | 16 | 99% | RA
08/04 08:15 | 98.4 | 80 | 116/70 | 16 | 99% | RA

LABS

07/31 | WBC 15.2 H | Hgb 12.8 | Plt 388 | Cr 0.7 | Glucose 104 | HbA1c 5.4%
08/03 | WBC 8.6 | Hgb 12.4 | Cr 0.7

MICRO

07/31 Blood cx x2: No growth, FINAL 08/04
07/31 Wound cx RLQ abdominal wall: Staphylococcus aureus, moderate growth. Susceptibilities: PENDING

IMAGING

07/31 CXR: lungs clear, no acute process

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Discharge medication list

DISCHARGE MEDICATION LIST

Patient: Rosa Doe MRN: SO-7724519

- Doxycycline 100 mg PO twice daily with food through 08/10/2026
- Acetaminophen 650 mg PO every 6 hours PRN pain
- Ibuprofen 400 mg PO every 8 hours PRN pain, with food

Wound supplies

- Sterile gauze 4x4, normal saline, paper tape (7-day supply)

Stopped

- Vancomycin IV (08/03)

Follow up

- General surgery clinic 08/12/2026 - Silver Oak Surgical Clinic (fictional) 555-0189

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